

Standard Operating Procedure

Labour and Delivery Monitoring

«Hospital Name» — Supports COP.10.g, COP.10.h

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» monitors a woman through labour and the immediate post-natal period, with particular attention to post-partum haemorrhage.

2. Scope

Applies to every patient in active labour and through the immediate post-natal period.

3. Responsibility

Labour Room Doctor/Nurse: monitors foetal heart rate, labour progression, and post-natal bleeding.

4. Procedure

1. Monitor foetal heart rate at the interval appropriate to the stage of labour and the patient's risk level.
2. Track labour progression against the hospital's adopted labour-monitoring tool (e.g. a partograph or the WHO Labour Care Guide).
3. Escalate immediately if foetal heart rate or labour progression falls outside the expected pattern.
4. After delivery, estimate blood loss and monitor for post-partum haemorrhage at defined intervals (e.g. at delivery, 1 hour, 2 hours).
5. Confirm uterine tone and bleeding are within expected limits before reducing monitoring frequency.

5. Stop-Work Authority

Do not continue routine monitoring frequency if foetal heart rate is abnormal or post-partum bleeding exceeds expected limits — escalate immediately.

Raising a concern about foetal heart rate or bleeding is never a disciplinary matter, regardless of seniority.

6. Records

Supported by Records 9 and 10 of the COP.10 Required Records pack.

7. Related Documents

- HCO_COP_10_v2_REWRITE_DRAFT — COP.10 policy

8. Review

Reviewed annually, and immediately after any maternal or neonatal adverse event.

9. References

- World Health Organization. (2018). WHO Recommendations: Intrapartum Care for a Positive Childbirth Experience. <https://www.who.int/publications/i/item/9789241550215> — the foetal monitoring and labour-progression principles in this SOP are consistent with this WHO guideline.